

The Access Barrier Report

The Cost of Getting the Benefit Alberta Then Takes Back — Documented with Primary Sources

A documented analysis of the step-by-step access barrier chain an AISH recipient must navigate to obtain the Canada Disability Benefit — from a documented case file held by the campaign, spanning 2019 to 2026.

The Alberta Disability System Breakdown — April 2026 Report Series | All figures sourced from Government of Canada, CRA, CPSA, and verified primary documents on record with the campaign

To access a benefit Alberta then claws back: a recipient must obtain an approved Disability Tax Credit. To obtain the DTC, a physician must complete a government form — at an unregulated, uncapped fee. After navigating all of this, the CDB pays \$200/month. Alberta deducts \$200/month. The net benefit to an AISH recipient: zero. The out-of-pocket cost to access that zero: documented at \$200 per adult application — unreimbursed.

The Canada Disability Benefit (CDB) was created by the federal government in 2024 as a poverty-reduction measure for working-age Canadians with disabilities. It pays up to \$200 per month. Alberta is the only province or territory in Canada to claw back this benefit dollar-for-dollar from AISH recipients.

This report documents not only the step-by-step access barrier chain a recipient must navigate to obtain the CDB before Alberta takes it back — it documents that chain with primary source evidence: timestamped government correspondence, official receipts, CRA account records, and Service Canada letters spanning 2019 to 2026. Every claim in this report is backed by a document held in a case file on record with the campaign.

SECTION 1: THE DEPOSIT STATEMENT TRAP

What the Government Said — and What It Left Out

AISH recipients were notified of the CDB requirement through their monthly direct deposit statements — standard-format letters mailed to recipients. The January 2026 and February 2026 deposit statements (confirmed via primary source on record with the campaign) contain the following language, buried in the body of the statement alongside income tax reminders and address-change notices:

“AISH CLIENTS MUST REPORT THE OUTCOME OF THEIR CANADA DISABILITY BENEFIT (CDB) APPLICATION BY FEBRUARY 28, 2026. IF NO DECISION HAS BEEN MADE BY THAT TIME, \$200 IN CDB INCOME WILL BE REDUCED FROM AISH BENEFITS STARTING IN APRIL 2026.”

— Government of Alberta AISH Direct Deposit Statement, January and February 2026 (Primary source: documented case file on record with the campaign)

That is the entirety of the communication. The deposit statement contains no further guidance. A recipient reading this notice for the first time would encounter the following:

What the statement says:

- Report the outcome of your CDB application by February 28, 2026.
- If no decision has been made, \$200 will be deducted from April 2026 benefits.

What the statement does not say:

- That a Disability Tax Credit (DTC) is a mandatory prerequisite to applying for the CDB.
- That a physician must complete Part B of the T2201 form before a DTC can be processed.
- That physicians in Alberta may charge a fee to complete this form with no regulated maximum.
- That this fee may range from \$75 to \$700 or higher depending on the physician.
- That 650,000 Albertans have no family doctor and may be unable to find a physician willing to complete the form.
- That CRA processing of DTC applications takes 8 to 12 weeks under normal conditions and was severely delayed due to the volume of simultaneous applications created by Alberta’s mandate.
- That if an applicant's SIN or identity information does not match CRA records, the CDB application will be administratively denied regardless of eligibility.
- That retroactive lump-sum CDB payments will also be deducted from AISH benefits.

*The deposit statement creates a compliance obligation with explicit financial consequences. It provides no pathway, no prerequisite information, and no guidance for the population it governs — 79,290 people, 28.3% of whom have cognitive disorders as their primary diagnosis. **This is not a communication gap. It is a communication design that maximizes non-compliance through inaccessibility.***

SECTION 2: THE COMPLETE CDB/DTC CHAIN — PRIMARY SOURCE DOCUMENTED

What the Access Barrier Looks Like from Inside a Single File

The following is the documented CDB/DTC access chain for one AISH recipient, drawn from a case file held by the campaign. Every entry in this timeline is supported by timestamped correspondence, official receipts, CRA account records, or Service Canada letters retained in the recipient's file.

DATE	EVENT	ACTION TAKEN / OUTCOME
Prior to 2025	Previous physician quoted \$700 to complete DTC forms	Fee unaffordable on AISH income. DTC was also functionally useless — it is a non-refundable tax credit with no benefit for people with no taxable income. Forms not completed. DTC allowed to lapse.
Prior to 2025	Previous physician retired	Complete medical file became inaccessible without payment of an excessive copying fee — itself an unregulated, uncapped cost. New physician inherited a patient with no transferred file.
June 1, 2025	New physician completes DTC forms — \$200 fee paid	Fee paid by debit at the treating clinic. Receipt retained. Reimbursement request submitted to AISH same day. Reimbursement not received as of April 2026. DTC submitted to CRA.
June 4–10, 2025	CRA receives and begins review of DTC application	CRA progress tracker confirms receipt June 4, 2025. Status updated to “In progress” June 10, 2025. Processing delay notification issued August 28, 2025: “ <i>significant increase in volumes.</i> ”

DATE	EVENT	ACTION TAKEN / OUTCOME
August 29, 2025	Recipient submits formal written account to AISH	Timestamped email documents full barrier chain in recipient's own words. Submitted to AISH proactively. Recipient's closing statement: <i>"Poor planning on your part does not constitute an emergency on mine."</i>
September 5, 2025	DTC approved — two periods	CRA My Account confirms: 2017–Indefinite (approved) and 2011–2016 (approved). One dependant child listed from 2017. Screenshot retained. CDB applied same day. Confirmation code retained on file.
September 5, 2025	AISH notified — DTC and CDB update	Email submitted with 4 attachments including both CRA progress trackers. Note included: <i>"The government needs to work on their clarity to avoid miscommunication."</i> Notified AISH within hours of approval.
September 12, 2025	CDB denied — administrative SIN mismatch	Service Canada letter (CDB-0008-003): unable to verify identity due to SIN/name/DOB mismatch. Not a denial of eligibility — an administrative processing failure. Deadline to respond: October 16, 2025.
September 26, 2025	SIN updated. Reapplied. Notified AISH.	SIN information corrected through Service Canada. Reapplication submitted same day. Email to AISH with attached documentation. Note: <i>"As always our government offices are stand alone with no cross communication."</i>

DATE	EVENT	ACTION TAKEN / OUTCOME
November 4, 2025	Second denial letter received — delayed by Canada Post strike	Letter dated September 12, 2025 arrived November 4, 2025 — 53 days after issue date, due to postal strike. No prior phone, email, or mail contact received requesting additional information. Reapplied same day. Noted: the CDB application asks only for name, DOB, SIN, address, citizenship, and general tax information. Unclear what was “missing or inaccurate.”
November 7, 2025	CDB approved	Service Canada letter (CDB-0003-005): eligible for benefit. \$1,000 lump sum (July–November 2025) deposited December 18, 2025. \$200/month December 2025 through June 2026.
December 4, 2025	CDB approval forwarded to AISH — same day received	Email with both pages of approval letter attached. Sent same day letter arrived.
February–March 2026	AISH deposits show \$2,206 — no communication about clawback	Both February 2026 (deposit date Jan 27) and March 2026 (deposit date Feb 24) statements show \$2,206. No letter, notice, phone call, or email from AISH explaining when the deduction will begin, how it will be calculated, or what the new net amount will be.
April 2026	Status of clawback: unknown to recipient	No communication received. Call centre wait times prohibitive. No individual caseworker to contact. Recipient has complied with every step. The government has not communicated what happens next.

*The entire chain — from the first physician fee to the approved benefit — cost the recipient \$200 out-of-pocket, required multiple applications across two government systems, involved an administrative denial caused by a government database mismatch, was delayed by a postal strike outside anyone's control, and ultimately resulted in Alberta clawing back every dollar of the approved benefit. **The deposit statement that started this chain mentioned none of it.***

SECTION 3: IN THE RECIPIENT'S OWN WORDS

Primary Source — August 29, 2025 Email to AISH

The following is drawn from a timestamped email submitted by the recipient to SCSS Edmonton AISH Mail on August 29, 2025 — before the DTC was approved, before the CDB was applied for, and before any deductions occurred. It documents the access barrier chain in real time, in the recipient's own words, submitted through official channels.

"As I need the DTC application approved first in order to apply for the CDB this is where it's at."

"I allowed my DTC to expire because; 1) I wasn't prepared to pay \$700 for my old family doctor to fill out the forms. And 2) The DTC did NOTHING for me without there being an employment income. So I let it be."

"Now I have a new doctor who isn't aware of all of my health issues due to my previous doctor going into retirement and my full medical file being unavailable without an excessive charge to have it copied. Which I am not in the financial position to pay for."

"However, my new doctor was able to fill out the new forms at the charge of \$200 and the application was submitted June 1st. Due to the backing up at the government level, the CRA office deadlines for approvals are taking longer. Which AISH should be aware of by now."

"Due to people trying to rush the process and applying for the CDB before having the DTC in place it's essentially crashing the system."

"If the DTC had been mandatory in the beginning much like the application of CPP was during the AISH application process this issue could have been avoided. AISH has a history of making radical changes while expecting its recipients to adapt and overcome barriers and unrealistic expectations."

"Poor planning on your part does not constitute an emergency on mine."

— The recipient, August 29, 2025 (Primary source: timestamped email to SCSS Edmonton AISH Mail, on record with the campaign)

This email was sent before any government deadline had passed, before any deduction had occurred, and before any formal advocacy was undertaken. It is a real-time account of a recipient attempting to comply with a mandatory process she had not been given adequate information to complete — and documenting exactly why.

SECTION 4: INACCESSIBLE COMMUNICATION DESIGN

The Elimination of the Human Translation Layer

Government communications to AISH recipients assume a level of administrative literacy, technological access, and physician availability that the recipient population does not uniformly possess. The problem is not only that the deposit statement omits critical information — it is that the mechanism previously available to bridge that gap no longer exists.

The Caseworker-to-Call-Centre Transition — Documented in a Single Inbox

From 2019 through 2024, AISH correspondence in this case file went directly to named workers with direct email addresses at the Edmonton AISH office. The recipient could reach her worker by name, escalate to a named supervisor, and receive individual responses within days. This is documented in a continuous email record on file with the campaign.

The transition point is January 14, 2025. The SCSS Edmonton AISH Mail address — the same address the recipient had been using for five years — returned the following auto-reply:

“Thank you for your email, this is an auto reply to acknowledge your message has successfully gone through and that it will be processed in the order it was received.”

“General Inquiries will not be responded to from this email.”

— SCSS Edmonton AISH Mail auto-reply, January 14, 2025 (Primary source: documented case file on record with the campaign)

This is the caseworker model ending — documented by the auto-reply itself. The individual worker who could read the file, exercise judgment, and communicate directly was replaced by a queue. The accountability that came with a named contact dissolved.

The Private Email Revelation — March 2023

A March 2023 exchange between the recipient and an AISH Generalist provides the most precise documentation of how the accountability structure had already begun to erode before the formal call-centre transition. When the recipient submitted documents to the worker’s direct email address, the Generalist responded:

“Good morning [recipient] — this e-mail is only for private documents within the government. This is not accessible to other AISH workers. If you send in a question to this e-mail and I am not in the office for days it will not be seen.”

“All documents are in order.”

— AISH Generalist, March 16, 2023 (Primary source: documented case file on record with the campaign)

The direct line the recipient had obtained through the public staff directory was not a true accountability mechanism. If the worker was absent, the email went unseen. No coverage, no escalation, no continuity. The call-centre transition formalized what this exchange reveals was already structurally true: the individual connection existed on paper but could not be relied upon.

Accessibility and the Recipient Population

The AISH caseload includes 28.3% of recipients with cognitive disorders as their primary diagnosis. It includes recipients with acquired brain injuries, autism spectrum disorder, severe depression, and psychotic disorders. The deposit statement — the primary vehicle for communicating compliance requirements with financial consequences — is:

- Written in standard bureaucratic English with no plain language alternative.
- Formatted identically to every other notice, with no visual urgency marking.
- Delivered by mail with no confirmation of receipt or comprehension.
- Unaccompanied by any caseworker call, follow-up, or explanation.
- Addressed to a population many of whom have no one to help them parse it.

The UNCRPD Article 9 (Accessibility) and Article 21 (Freedom of Expression and Access to Information) require that information be provided in accessible formats and through accessible means. The Alberta Human Rights Act requires reasonable accommodation. A compliance deadline buried in a deposit statement does not meet these standards for a population defined by severe and permanent disability.

SECTION 5: THE COMPLIANCE RECORD

Counter-Evidence to the Narrative That Recipients Fail to Comply

The following documents every compliance action taken by the recipient in this case file between 2019 and 2026. Every entry is supported by a timestamped document. Entries marked **(Recipient)** indicate actions initiated proactively without prompting. Entries marked **(Government Request)** indicate government-initiated requests to which the recipient responded.

DATE	ACTION	INITIATED BY
Jan 30, 2019	Proactive notification of file transfer from prior office. Voicemail and email sent to incoming worker before transfer completed.	Recipient
Feb 4, 2019	Landlord confirmation letter submitted — lease, damage deposit, payment schedule. Submitted same week as move-in.	Recipient
Apr 17, 2019	2018 Notice of Assessment submitted. Filed and sent same month available from CRA.	Recipient
Jun 1, 2021	June AISH benefit missing. No letter, notice, or call received. Recipient discovered on cheque line. Emailed worker directly, CC'd supervisor, notified MLA — all same day. Reattached prior notice of assessment proactively. Cause: AISH had outdated phone number on file despite update over one year prior.	Recipient
Aug 10, 2021	AADL equipment request for a dependent — found more affordable option independently. Submitted update proactively.	Recipient
Aug 28, 2021	Chiropractic evaluation results submitted with documentation. Clinical finding: twisted hips and shoulder strain significantly impairing physical capacity. Submitted unprompted.	Recipient
Mar 9, 2022	Glasses missing for over a month. Migraines worsening. Requested coverage for second pair. Gas and power bill increased significantly. Cost of groceries and children's clothing increased. Notice of assessment attached proactively.	Recipient

DATE	ACTION	INITIATED BY
Mar 14, 2022	Eye appointment follow-up — notified AISH that new glasses cannot be scheduled until later in year due to provider scheduling.	Recipient
Jan 30, 2023	Queried whether affordability supplement applies to dependent children under same file number. Government response: does not include dependents.	Recipient
Mar 16, 2023	Bank statements submitted — two accounts. Complete written explanation: no bonds, no savings, no income beyond AISH and child tax. Confirmed all transactions.	Government Request
Dec 19, 2023	Dental and health update submitted. Documented one year of oral infections, liquid diet, tooth loss, multiple emergency dental visits. Long-term immune condition diagnosed 2018. Cannot have general anesthesia. Surgery scheduled. Requested support for procedure.	Recipient
Jun 5, 2024	Vision emergency reported — halos and prisms observed during night driving. Possible major prescription change, glaucoma/cataract screening required. Coverage not refreshing for several months. Requested emergency coverage authorization.	Recipient
Mar 6, 2025	Cost-of-living summary submitted. Documented grocery, fuel tax, electricity, and rent increases. Requested discussion of live-in support arrangement to offset costs. Original email sent January 2025. Resent March 6 with note that no response had been received.	Recipient

DATE	ACTION	INITIATED BY
Jun 24, 2025	DTC medical fee receipt (\$200) submitted. Reimbursement requested. Never reimbursed.	Recipient
Jul 2–24, 2025	Account status request from AISH. Full itemized financial breakdown submitted same day: AISH, GST, child tax, rental subsidy, all accounts explained. No response for over a week. MLA constituency assistant emailed ministry directly to expedite.	Government Request
Aug 29, 2025	Formal written account of full DTC/CDB access barrier chain submitted to AISH — timestamped email. Documented all barriers including physician fee, expired DTC, inaccessible medical records, CRA processing delays.	Recipient
Sep 3, 2025	Medical transportation reimbursement — August 2025. Twenty attachments. Eight separate trips itemized: psychology, dental, mental health, hospital ×2, provincial lab, OT, women's health, pediatric.	Recipient
Sep 5, 2025	DTC approval and CDB application submitted same day. CRA My Account screenshot retained. All attachments included.	Recipient
Sep 26, 2025	SIN update completed. CDB reapplication submitted same day. AISH notified with documentation.	Recipient
Nov 4, 2025	Second CDB denial letter received (delayed by Canada Post strike). Reapplied same day. AISH notified.	Recipient
Dec 4, 2025	CDB approval letter forwarded to AISH — same day received.	Recipient

DATE	ACTION	INITIATED BY
Jan 31, 2026	2026–2027 lease renewal submitted proactively.	Recipient
Mar 29, 2026	Executive Summary and Personal Impact Statement submitted — signed and dated — to Premier, Minister, MLA, AISH/ADAP, and Alberta Ombudsman.	Recipient

This file has been continuously compliant for ten years. Every document request was answered promptly. Every change in circumstances was reported. Every annual notice of assessment was submitted the month it became available. Every lease renewal was filed proactively. Every government error — the wrong phone number, the held cheque, the missing file transfer — was identified, corrected, and documented by the recipient, not the government.

SECTION 6: THE ORIGINAL ACCESS CHAIN — STEP BY STEP

The Documented Sequence Every AISH Recipient Must Complete

The following is the documented sequence a person on AISH must complete to receive the Canada Disability Benefit. Each step is a barrier. For a population defined by permanent, severe disability — many of whom have no family doctor, no transportation, and limited administrative capacity — each barrier is a point of failure.

Step 1 — Apply for the Disability Tax Credit

Before a person can apply for the CDB, they must hold an approved Disability Tax Credit (DTC) certificate from the Canada Revenue Agency. The DTC is a non-refundable federal tax credit. There is no alternative gateway to the CDB.

- The DTC application form (T2201) has two parts: Part A is completed by the applicant; Part B must be completed by a qualified medical practitioner.
- The CRA processes DTC applications in approximately 8 to 12 weeks under normal conditions. At high-volume periods — such as Alberta’s mandatory mass application in 2025 — this extends significantly.
- Forcing 79,290 AISH recipients to apply for the DTC simultaneously, under threat of a \$200 deduction, created the processing backlog that then generated the compliance failures Alberta used to justify those deductions. The system created its own crisis.

*AISH recipients are legally required to apply for all federal programs they may be eligible for — including the CDB. **Failure to complete this process by February 28, 2026 resulted in an automatic \$200 deduction from April 2026 AISH benefits.** Recipients were being penalized for not completing a process that depends on physician availability, physician willingness, application processing time, and CRA approval — none of which are within a recipient's control.*

Step 2 — Obtain Physician Completion of the Form

Part B of the T2201 form must be completed by a physician or other regulated health professional. This creates two intersecting barriers.

Barrier A: You must have a physician.

As documented in the Healthcare Worker Shortage Report, 650,000 Albertans have no family doctor. Family practices accepting new patients collapsed from 887 in 2020 to 164 in 2024. Walk-in physicians will frequently decline to complete complex disability forms for patients they have seen once.

Barrier B: Physicians may charge a fee to complete the form.

The College of Physicians & Surgeons of Alberta (CPSA) confirms that physicians may bill patients directly for services not covered by Alberta Health Care — explicitly including the completion of government and insurance forms. There is no regulated maximum fee in Alberta.

DOCUMENTED FEE INSTANCE	AMOUNT
St. Albert clinic, June 2025 — primary source receipt on record with the campaign, paid by debit	\$200.00
GP completing T2201 for adult AISH recipient — St. Albert clinic, 2024 (documented invoice, no prior price agreement)	\$520 billed — reduced to \$135 after dispute
Previous physician quoted to complete T2201 — Edmonton area, prior to 2025	\$700 quoted — not completed

Step 3 — Submit the DTC Application and Wait

Once Part B is completed and payment made, the T2201 is submitted to CRA. Processing takes 8 to 12 weeks under normal conditions. Alberta's mandatory mass application effectively crashed the CRA processing system, extending timelines significantly. Recipients who submitted early — before the system was overloaded — received approvals. Recipients who submitted later faced delays they did not cause and could not control, and were then penalized for those delays by Alberta's February 28, 2026 deadline.

Step 4 — Apply for the CDB

Once a DTC is approved, the recipient may apply for the Canada Disability Benefit through Service Canada. The CDB application asks for name, date of birth, SIN, address, citizenship status, and general tax information. If any of this information does not match CRA records — including a SIN that was previously issued under different information — the application will be denied on administrative grounds regardless of eligibility. Reapplication resets the 28-calendar-day processing window.

Step 5 — Alberta Claws Back the Benefit

Once approved, the CDB pays up to \$200/month. Alberta treats this as income and deducts it dollar-for-dollar from AISH benefits. The net financial benefit to an AISH recipient who successfully navigates all four preceding steps: zero. The cost of accessing that zero: documented at \$200 or more per adult application.

SECTION 7: WHAT IS BEING ASKED

To the Government of Alberta

- Immediately cease the dollar-for-dollar clawback of the Canada Disability Benefit. Alberta is the only province or territory in Canada to do this. Every dollar of the CDB received by an AISH recipient should remain with that recipient.
- Retroactively reimburse AISH recipients for the \$200 deducted from April 2026 benefits where the deduction was caused by CRA processing delays outside the recipient's control.
- Reimburse all documented DTC physician fees incurred by AISH recipients in compliance with the government's own mandate. The reimbursement commitment was made. It must be honoured.
- Redesign all compliance communications to meet plain language accessibility standards. Every AISH compliance notice must state the complete prerequisite chain, the associated costs, and the pathway — not only the deadline and the consequence.
- Restore individual caseworker access for AISH recipients. The call-centre model eliminates accountability and disproportionately harms recipients with cognitive, psychiatric, and communication-related disabilities.

To the Government of Canada

- Legislate that the Canada Disability Benefit cannot be used as a provincial income offset. Alberta's clawback directly nullifies the federal poverty reduction intent of the benefit. A non-binding call to provinces is not sufficient.
- Accelerate the proposed \$150 supplemental CDB payment to offset DTC certification costs. As of April 2026 this payment has not been made.
- Review the DTC gateway requirement for CDB eligibility in provinces that claw back the benefit in full. Where a province eliminates the net benefit, the DTC application process imposes costs and burdens with no corresponding benefit to the recipient.

- Require provincial non-clawback compliance as a condition of federal disability benefit participation — not as a recommendation.

SOURCES AND REFERENCES

Primary Sources — Documented Case File on Record with the Campaign

- Timestamped email correspondence between the recipient and SCSS Edmonton AISH Mail / CSS Edmonton AISH, 2019–2026. Includes: file transfer notification (Jan 30, 2019), landlord confirmation (Feb 4, 2019), notice of assessment submissions (2019–2025), medical result submissions (2021–2023), chiropractic evaluation (Aug 2021), dental update (Dec 2023), vision emergency (Jun 2024), cost-of-living submission (Jan/Mar 2025), DTC reimbursement request (Jun 24, 2025), account status disclosure (Jul 2025), August 29, 2025 access barrier account, medical transportation reimbursement with twenty attachments (Sep 3, 2025), DTC/CDB update (Sep 5, 2025), CDB reapplications (Sep 26, Nov 4, 2025), CDB approval forwarded (Dec 4, 2025), lease renewal (Jan 31, 2026).
- Direct deposit statements — Government of Alberta AISH, January 2026 (deposit date Jan 27, 2026) and March 2026 (deposit date Feb 24, 2026). CDB compliance notice included.
- Receipt for DTC completion — St. Albert medical clinic, June 24, 2025. \$200.00 paid by debit.
- CRA My Account screenshot — September 5, 2025. DTC approved 2017–Indefinite and 2011–2016. One dependant child listed.
- DTC progress trackers — Application 80657091 (received Jun 4, delayed to Sep 26) and 81257854 (received Jun 24, completed Sep 5, 2025).
- CDB confirmation screenshot — Application successfully submitted September 5, 2025.
- Service Canada CDB denial letter — September 12, 2025. CDB-0008-003. SIN/identity mismatch. Administrative, not eligibility.
- Service Canada CDB approval letter — November 11, 2025. CDB-0003-005. Approved November 7, 2025. \$1,000 lump sum December 18, 2025. \$200/month December 2025–June 2026.
- MLA Marie Renaud constituency office correspondence — July 2025. Confirms direct intervention with ministry to expedite response to account status request.
- AISH Generalist email — March 16, 2023. Confirms personal email not accessible to other AISH workers; documents communication limitation of direct-worker model.
- SCSS Edmonton AISH Mail auto-reply — January 14, 2025. Confirms transition to call-centre model. States: *“General Inquiries will not be responded to from this email.”*

Government and Institutional Sources

- Government of Alberta — AISH Direct Deposit Statement format. AISH CDB/DTC notification, January and February 2026.
- College of Physicians & Surgeons of Alberta (CPSA) — Physician billing for form completion. cpsa.ca/faqs. No regulated maximum identified.

- Canada Revenue Agency — Disability Tax Credit, T2201 form and eligibility criteria. canada.ca
- Government of Canada — Canada Disability Benefit program. Service Canada. Application process, eligibility, and payment schedule. canada.ca/en/services/benefits/disability/canada-disability-benefit.html
- Government of Alberta — AISH Caseload Open Data, September 2025. 79,290 recipients; 28.3% cognitive disorder primary diagnosis; 83.6% zero employment income.
- Healthcare Worker Shortage Report — companion document. 650,000 Albertans without a GP; 887 to 164 accepting practices (2020–2024).
- Financial Reality Report — companion document. Alberta is the only province or territory in Canada to claw back the Canada Disability Benefit dollar-for-dollar from disability recipients.
- United Nations Convention on the Rights of Persons with Disabilities (UNCRPD) — Articles 9 (Accessibility), 21 (Freedom of Expression and Access to Information), 19 (Independent Living). Canada ratified 2010.

The Alberta Disability System Breakdown — Advocate

St. Albert, Alberta | April 2026

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